

1. PATIENT INFORMATION

- **Name:** John D. Miller
- **Age:** 58
- **Gender:** Male
- **Date of Visit:** January 10, 2026
- **Medical Record Number (MRN):** 8842-GH-1092

2. CHIEF COMPLAINT & SYMPTOMS

- **Primary Symptoms:** Sudden onset of severe substernal chest discomfort. The patient describes the sensation as a "crushing weight" located behind the sternum.
- **Duration and Severity:** Symptoms began 90 minutes prior to admission. Pain severity is 9/10 and is unresponsive to changes in position or deep inspiration.
- **Associated Symptoms:** Discomfort radiates into the left cervical region and the left upper extremity. Significant autonomic distress is noted, including nausea and profuse diaphoresis.

3. MEDICAL HISTORY

- **Past Medical Conditions:** Chronic hypertension, hyperlipidemia, and Type 2 Diabetes Mellitus.
- **Current Medications:** Lisinopril 20mg, Metformin 1000mg, Atorvastatin 40mg.
- **Allergies:** Penicillin (Urticaria).

4. PHYSICAL EXAMINATION

- **Vital Signs:**
 - **BP:** 162/98 mmHg
 - **HR:** 112 bpm (Sinus Tachycardia)
 - **Temp:** 37.1°C
 - **RR:** 24 breaths/min
 - **O2 Sat:** 91% (Room Air)
- **Physical Findings:** Patient is acutely distressed, diaphoretic, and pale. Auscultation of the lungs reveals fine bibasilar inspiratory crackles. Cardiac exam reveals a prominent S4 gallop at the apex; no S3 or murmurs noted. JVP is slightly elevated at 4cm above the sternal angle.

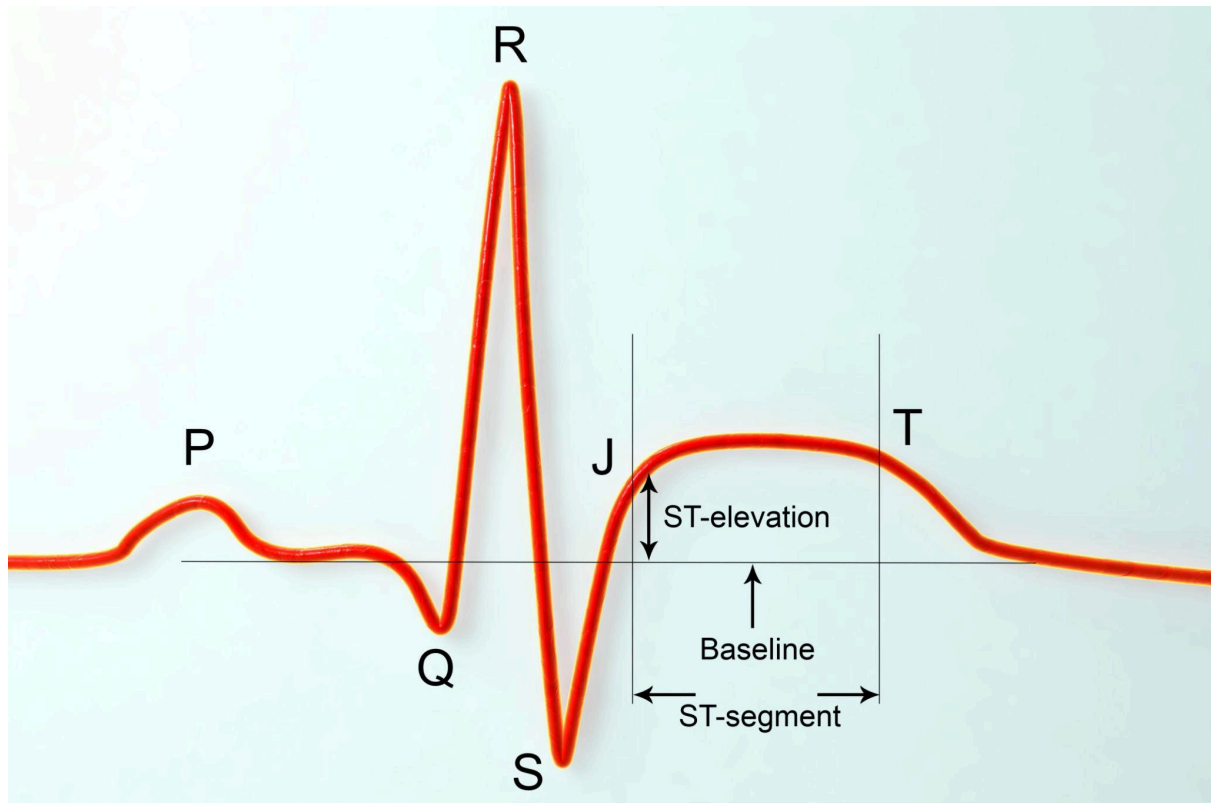
5. LABORATORY RESULTS

Test	Result	Normal Range	Interpretation

Hemoglobin	14.2 g/dL	13.5 - 17.5 g/dL	Within Normal Limits
WBC Count	11.5 x 10^9 /L	4.5 - 11.0 x 10^9 /L	Elevated
Glucose (Random)	185 mg/dL	70 - 140 mg/dL	Hyperglycemic
Creatinine	1.1 mg/dL	0.7 - 1.3 mg/dL	Within Normal Limits
Troponin T	1.45 ng/mL	< 0.01 ng/mL	Critical Elevation
CK-MB	28 ng/mL	< 5 ng/mL	Critical Elevation

6. DIAGNOSTIC STUDIES

- **12-Lead Electrocardiogram (ECG):** Evidence of significant ST-segment elevation in the precordial leads (V1-V4), with reciprocal ST-depression in the inferior leads.



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- **Portable Chest Radiograph:** Findings include mild cephalization of the pulmonary vessels and interstitial edema.
- **Transthoracic Echocardiogram:** Findings indicate a Left Ventricular Ejection Fraction (LVEF) of 40% with regional wall motion abnormalities (hypokinesis) in the anterior and apical segments.

7. CLINICAL NOTES & ASSESSMENT

Assessment: A 58-year-old male presenting with acute-onset ischemic chest pain and evidence of severe myocardial cell injury as indicated by serum biomarkers. The electrical activity on the ECG combined with the regional wall motion abnormalities suggests a localized area of acute tissue hypoxia and necrosis in the territory of the Left Anterior Descending artery.

Plan: 1. Initiation of antiplatelet and anticoagulation therapy.

2. Immediate activation of the Cardiac Catheterization Team for emergent revascularization.

3. Continuous telemetry and pulse oximetry monitoring.